

Clinical Criteria for Common Procedures

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1. Purpose

This document provides evidence-based clinical criteria for evaluating the medical necessity of common procedures.

2. Cardiovascular Procedures

2.1 Echocardiogram (CPT-93306)

Indications:

- Suspected heart failure (SOB, edema, fatigue)
- Evaluation of valvular disease (murmur, known valve pathology)
- Assessment of cardiac function post-MI
- Evaluation of cardiomyopathy
- Pre-operative assessment (high-risk surgery)

Approval Criteria:

- Documented symptoms (dyspnea, chest pain, palpitations)
- Prior EKG abnormalities
- Age ≥ 65 with cardiovascular risk factors
- Known cardiovascular disease with change in symptoms

Exclusions:

- Routine screening in asymptomatic patients
- Annual monitoring of stable conditions
- Repeat echocardiography within 90 days without clinical change

2.2 Electrocardiogram (CPT-93000)

Indications:

- Chest pain or anginal symptoms
- Pre-operative assessment (cardiac risk)
- Palpitations or arrhythmia evaluation
- Syncope evaluation
- Baseline EKG for new medications

Approval Criteria:

- Symptoms of cardiac origin
- Physician order with clinical rationale
- Pre-operative for surgery with cardiac risk

Exclusions:

- Routine screening without symptoms
- Annual EKG without indication
- Repeat EKG within 30 days without clinical change

3. Gastrointestinal Procedures

3.1 Upper GI Endoscopy (CPT-43239)

Indications:

- Upper GI bleeding (hematemesis, melena)
- Dysphagia (difficulty swallowing)
- Persistent GERD (≥ 3 months)
- Iron deficiency anemia
- Suspected malignancy

Approval Criteria:

- Documented GI symptoms
- Prior non-invasive testing (if applicable)
- Failure of medical therapy

Exclusions:

- Routine screening in asymptomatic patients
- Annual monitoring without clinical change

3.2 Colonoscopy (CPT-45378)

Indications:

- Colorectal cancer screening
- Rectal bleeding
- Change in bowel habits
- Iron deficiency anemia
- Inflammatory bowel disease monitoring

Approval Criteria:

- Age ≥ 50 for screening (or earlier with risk factors)
- Documented symptoms
- Family history of colorectal cancer

Exclusions:

- Routine screening more frequently than every 10 years without indications

4. Orthopedic Procedures

4.1 Lower Extremity MRI (CPT-73720)

Indications:

- Suspected ligament/meniscus tear
- Joint pain not responsive to conservative therapy
- Suspected fracture (not visible on X-ray)
- Evaluation of mass or lesion

Approval Criteria:

- Documented pain or injury
- Prior X-ray performed
- Conservative therapy failed (≥ 6 weeks)

Exclusions:

- Initial evaluation of acute injury without X-ray
- Routine monitoring without clinical change
- MRI without prior X-ray

Clinical criteria adapted from evidence-based guidelines. Prior authorization may be required as noted in HLT-PROD-01.